Gift, risk and trust

Ethical problems in organ transplantation and in research involving humans

Practical Class 6 · Social Medicine and Medical Ethics I · 2026/2027

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Five questions for today's class

1. Transplantation: concepts and death
2. Ethical problems in transplantation
3. Transplantation in Bulgarian law
4. Ethics of research involving humans
5. Research in EU and Bulgarian law

Transplantation: concepts and death

Transplantation: definition and types

Type	Source of the graft	Typical ethical question
Autotransplantation	The same person	Consent and proportionate risk
Isotransplantation	Identical twin	The donor's voluntariness
Allotransplantation	Another person, living or deceased	Consent, fair allocation, no payment
Xenotransplantation	Animal	Infection risk to the public, follow-up, treatment of animals

Transplantation of Organs, Tissues and Cells Act, Art. 2: removal from a cadaver or a living person and implantation in another person **for treatment**.

The donor: living or deceased

Living donor

One of paired organs, part of a regenerating organ, tissues and cells. Surgery on a healthy person with no direct bodily benefit.

Brain death

Irreversible loss of all brain functions while the heart still beats. **Not a coma**.

Circulatory death (DCD)

After irreversible cessation of circulation. The decision to withdraw treatment is independent of donation.

Death is determined by the same criteria for every patient, whether or not donation is possible.

The dead donor rule

What it requires

A vital organ is removed only after death has been properly determined. Removal does not cause death.

How the law secures it

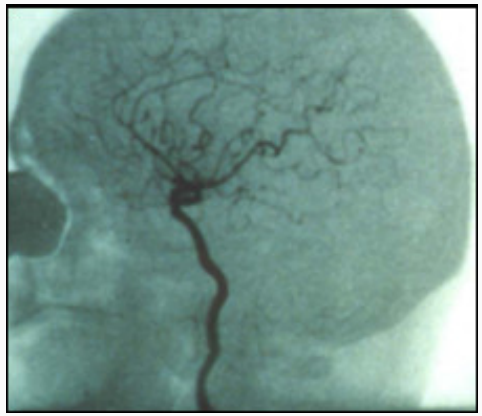
A committee of three physicians appointed with IAMN's consent (Transplantation Act, Art. 18(2)); they take no part in removal or transplantation (Art. 18(3)).

"If I agreed to donate with a donor card, will the doctors still do everything possible to save my life after an accident?"

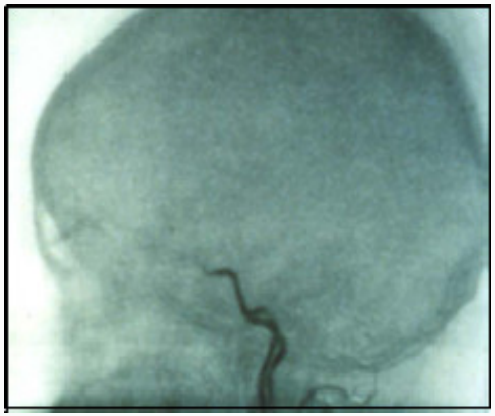
"Doctors do their utmost to save human life, because this is their moral duty and right."

Source: Bulgarian Executive Agency for Medical Supervision (IAMN), iamn.bg, FAQ on transplantation and donation (translated)

Brain death: how it is determined



A) Cerebral angiography — cerebral blood flow **present**



B) Cerebral angiography — cerebral blood flow **absent**

Source: Bulgarian Executive Agency for Medical Supervision (IAMN), iamn.bg, FAQ on transplantation and donation

BRAIN DEATH

No spontaneous breathing · Irreversible loss of brain function, including the brainstem · No cerebral blood flow

- Committee of three physicians (intensive care, neurology, neurosurgery, radiology)
- Not started in hypothermia, intoxication or under muscle relaxants
- Two clinical examinations ≥ 12 hours apart, or with EEG / a blood-flow test
- Blood-flow test mandatory in children under 6

Ordinance No. 14 of 15 April 2004, Arts. 8–14

Ethical problems in transplantation

Consent models for deceased donation

Opt-in (explicit consent)

Only those who agreed in life are donors. Respects an active choice; yields fewer donors.

Opt-out (presumed consent)

Donation is allowed unless the person objected. Ethical only if citizens are informed and objecting is easy.

Bulgaria — presumed consent with safeguards: a written objection made in life and entered in the IAMN register (Transplantation Act, Arts. 19–20); relatives must be informed, and their written refusal stops removal — first spouse or parent, then children, then siblings (Art. 21); under 18, only with the parents' written consent (Art. 19(2)).

How a citizen registers an objection

Insured, with a GP

Standard declaration signed before the **GP** → Entered in the health insurance booklet; within 7 days to the **regional health inspectorate** → Within 7 days the inspectorate informs **IAMN** → Within 3 days: entered in IAMN's **official register**

No GP or no health insurance

Declaration at the **municipality** of permanent address, in 2 copies → Within 7 days the municipality sends a copy to **IAMN** → Entered in IAMN's **official register**

The objection may cover all or specific organs and purposes (para 2). Physicians can check the register 24/7. **2,858 people** registered an objection from 2005 to 6 Oct 2026.

Transplantation Act, Art. 20; declaration form (Minister's Order No. RD-01-387/10.08.2022) and statistics: IAMN, iamn.bg

Case 1 · The wish expressed in life

A 24-year-old man has been declared brain dead after a road accident. The register shows no objection, and there is a donor card in his wallet. His parents confirm he supported donation. When the procedure becomes real, his mother cannot bear it and his father submits a written refusal.

- What is the moral weight of the donor card, and what is its legal weight?
- What follows from Art. 21 of the Transplantation Act once the father refuses in writing?
- How does the team talk to the parents without pressure?

The living donor is a patient in their own right

Voluntariness

Notarised consent (Transplantation Act, Art. 24(1)); informed by a physician outside the team (para 3); may withdraw any time before removal (para 4); interview alone.

Protection from risk

No danger to life (para 1); committee of three physicians outside the team (para 7); long-term follow-up.

Who may donate

Adult spouse or relative up to the fourth degree (Art. 26(1)); exceptions with the Transplantation Ethics Committee's permission (para 2).

No payment

Material benefit prohibited (Art. 24(8)); only compensation for costs and lost income (para 9).

Children and paired exchange

A child as donor

Organs from persons under 18: never (Transplantation Act, Art. 24(5)). Regenerating tissue and bone marrow: narrow exceptions with the Transplantation Ethics Committee's permission (Arts. 27, 27a).

Paired exchange

Two incompatible pairs swap donors. Removal and implantation are **simultaneous** (Art. 26a(2); Art. 33a).

A child is not a family tissue reserve, and a gift does not become a debt.

Fair allocation of scarce organs

Acceptable criteria

- Compatibility
- Medical urgency
- Expected benefit
- Waiting time

Unacceptable criteria

- Wealth, fame, media pressure
- Ethnicity, sex, occupation
- “Social worth”, moral judgement

Kidney (Ordinance No. 17, Annex 1): blood group → HLA match → sensitisation → negative crossmatch → clinical condition → waiting time. Children have priority for kidneys from donors under 35. IAMN allocates organs (Transplantation Act, Art. 11(5)(6)).

The organ’s path: who decides what

1 The hospital

Identifies a potential donor in intensive care; the donation coordinator notifies IAMN.

2 Three-physician committee

Determines death; takes no part in transplantation (Art. 18).

3 Register and relatives

IAMN register checked; relatives must be informed (Arts. 19–21).

4 The donor

Donor management, tests under the medical standard, donor file.

5 IAMN committee

Ranks recipients by the algorithm, with a patient representative; the IAMN director approves.

6 Transplant centre

Accepts within 4 hours, or the organ goes to the next recipient; failing that, exchange abroad.

Transplantation Act, Arts. 18–23; Ordinance No. 17 of 2004, Arts. 9a–11; donor file and organ exchange (Eurotransplant, FOEDUS, Romania): IAMN, iamn.bg

Trade, trafficking and transplant tourism

Ethical standard

The Declaration of Istanbul (2008, rev. 2018) and the WHO principles: no payment, transparency, national self-sufficiency in organs.

Bulgarian law

Transplantation Act: organs are not traded (Art. 5); no benefit may be offered (Art. 6). Criminal Code: trafficking for organ removal (Art. 159a); breaching the removal rules (Art. 349a).

A bought organ exploits a poor donor. Treating the returning patient remains a duty.

Case 2 · The “other” procedure

> A 48-year-old man on haemodialysis has waited a year for a kidney. He tells his nephrologist that through a broker he has arranged a transplant at a private clinic outside the EU; the price includes “providing a donor”. He asks for an immunosuppressive regimen and follow-up after his return, in full confidence.

- How does this differ from lawful treatment abroad?
- What does the physician owe him before he travels?
- Does the duty of care end after his return?

Transplantation in Bulgarian law

Transplantation of Organs, Tissues and Cells Act

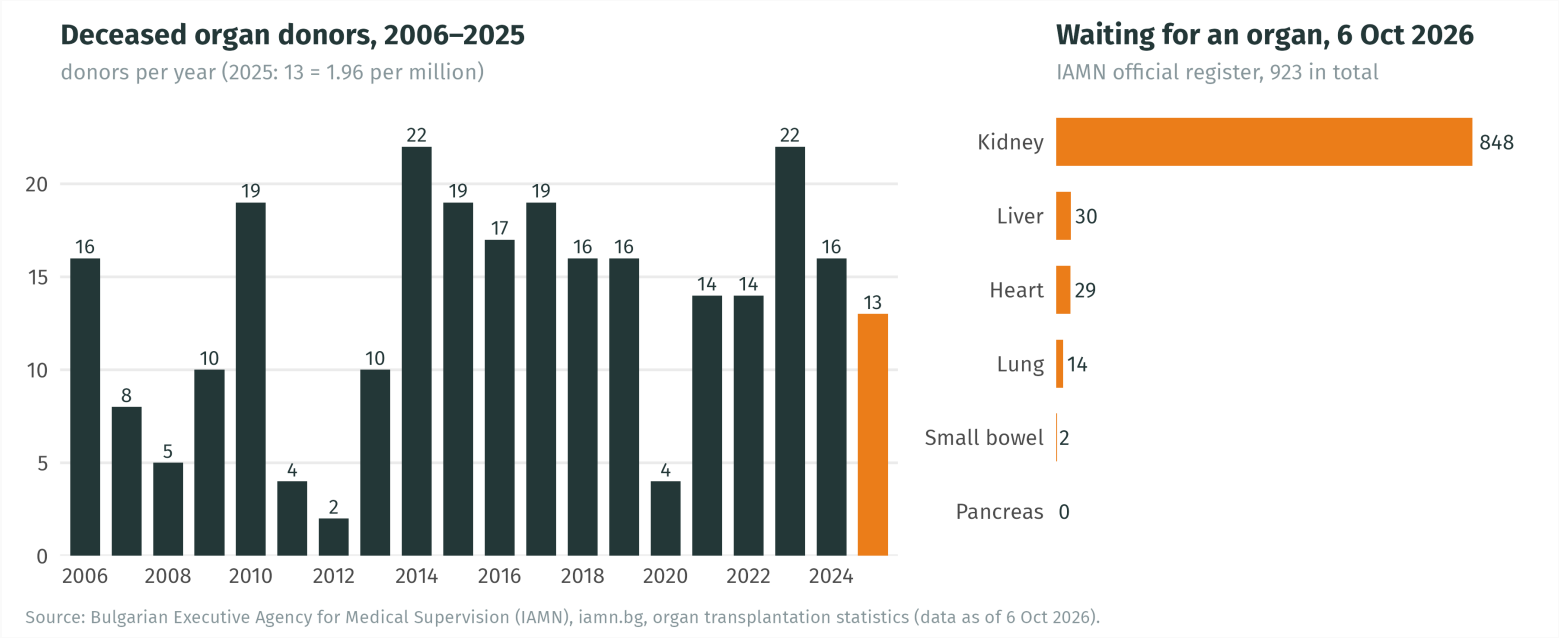
Article	Content
Arts. 3, 5, 6	Equal rights of patients; organs not traded; no benefit or advertising
Art. 8	Donor and recipient identities may not be disclosed
Art. 11(4)	Medical Supervision Agency manages, coordinates and controls transplantation
Art. 12	Transplantation Ethics Committee at the Council of Ministers
Arts. 18–21	Death, objection in life, informing relatives
Arts. 24–27a	Living donor: consent, who may donate, children, paired exchange
Arts. 32–33, 39	Recipient register, selection, public and official registers

Ordinances and institutions

Act	Subject
Ordinance No. 14 / 15.04.2004	Medical criteria and procedure for determining death, including brain death
Ordinance No. 12 / 4.04.2007	Establishing the conditions for removal from a deceased person (register, relatives)
Ordinance No. 17 / 2004	IAMN's official register of patients waiting; selection of the recipient
Ordinance No. 6 / 5.03.2007	Medical standard for transplantation: donors, tests, traceability
Ordinance No. 13 / 6.12.2021	Reimbursement by the Ministry of Health, including the living donor's costs
Ordinance No. 2 / 27.03.2019	Treatment and transplantation abroad; approved and paid by the NHIF

Management, coordination and control: Executive Agency for Medical Supervision (IAMN, iamn.bg).
Public information: the "Yes! For Life!" portal (zajivot.bg).

Transplantation in Bulgaria in numbers



2026 up to 6 Oct: **13 donors, 26 organ transplants**. Since 2005, transplantation has saved **1,921 lives** (IAMN).

Xenotransplantation: the limits of innovation

Promise

Genetically modified animals as an organ source against the shortage (first pig heart in a human, 2022).

Problem

A new infection would reach relatives and the public; one person’s consent cannot cover that.

Innovation needs scientific justification, independent ethical review and long-term follow-up.

Ethics of research involving humans

History as a lesson

Year	Event / document	Lesson
1947	Nuremberg Code	Voluntary consent is essential; the right to stop
1932–1972	Tuskegee, USA	Deception and withheld treatment in a dependent group
1964 / 2024	Declaration of Helsinki	Ethics committee, vulnerability, registration, post-trial care
1966	Beecher, NEJM	Unethical studies in democracies too
1979	Belmont Report	Respect for persons, beneficence, justice
2016	CIOMS guidelines	Fairness in low-resource settings, community engagement

When is research ethical?

1 Social value	**2** Scientific validity	**3** Fair selection	**4** Risk–benefit
5 Independent review	**6** Informed consent	**7** Respect for participants	

Health Act, Art. 197(3) and (5): the participant has all the rights of a patient; their interests outweigh the researcher's scientific and financial interests.

Poor design is an ethical problem: the risk stays, the benefit disappears.

Informed consent to take part

What the participant must know

Purpose and experimental nature; randomisation and placebo; risks and benefits; alternatives; data; compensation; withdrawal without losing care.

Therapeutic misconception

The belief that the protocol was chosen for my treatment. The researcher separates care from science and checks understanding.

Health Act, Art. 199: written consent after written information; given in person; revocable at any time.

Vulnerability, randomisation and placebo

Vulnerable participants

Children, adults lacking capacity, pregnant women, prisoners, students and employees, dependent patients. Health Act, Art. 200: no research on persons lacking legal capacity; pregnant women and prisoners only with significant expected benefit to them.

Randomisation and placebo

Randomisation requires clinical equipoise. Placebo only when no proven treatment exists, or without risk of serious or irreversible harm (Helsinki 2024).

Ethics committees and Good Clinical Practice

Ethics committee

Independent, with lay members. Reviews before the start and monitors during the study.

Sponsor

Organises, funds, monitors; insures; liable when the protocol was followed.

Principal investigator

Conducts the study at the site; consent; medical decisions; records.

Good Clinical Practice protects both the participants and the reliability of the data.

Case 3 · “This is the newest treatment”

> An oncologist invites his patient into a phase II clinical trial. He calls the product “the newest treatment” and stresses the more frequent scans, but does not say its effect is unproven. The patient signs and tells his family the doctor chose the best for him. The oncologist is paid for each participant enrolled.

- Is this therapeutic misconception? How is it corrected?
- Which interests must be declared?
- Should consent be obtained again?

Research in EU and Bulgarian law

Clinical trials of medicinal products

Regulation (EU) No 536/2014

Single submission through CTIS; applies from 31.01.2022, to all trials from 31.01.2025.

Medicinal Products in Human Medicine Act

Authorisation by the Bulgarian Drug Agency (Arts. 81, 84); ethical opinion by the Ethics Committee for Clinical Trials at the Minister of Health (Arts. 83, 103).

Consent

Written (Art. 96); for young children both parents, for adolescents also the adolescent (Art. 97). A child's refusal is respected.

Liability

Insurance (Art. 91); the sponsor if the protocol was followed, the investigator if not (Art. 92).

Medical research under the Health Act

Article	Content
Art. 197	The participant has patient rights; their interests come first
Art. 198	Prohibited: no evidence of safety, increased risk, changes to the genome
Art. 199	Written, personal, revocable informed consent
Art. 200	Not on persons lacking capacity; pregnant women and prisoners only with significant benefit
Art. 201	Joint liability of the study lead and team; compulsory insurance
Art. 203	Positive opinion of a local ethics committee in the institution

Two regimes, two committees

	Clinical trial of a medicinal product	Other medical research
Law	Regulation (EU) No 536/2014; Medicinal Products Act, Arts. 81–107a	Health Act, Arts. 197–205
Authorisation	Bulgarian Drug Agency via CTIS	—
Ethical review	Ethics Committee for Clinical Trials at the Minister of Health	Local ethics committee in the institution
Consent	Written; special rules for children and adults lacking capacity	Written, in person; persons lacking capacity excluded

A drug study on humans is a **clinical trial** (Bulgarian: „клинично изпитване“).

Gifts and participation are voluntary.

The rules exist so that trust survives.